









TEAM PROMPT!PLEASE

NIMISH JAIN | ETANT RUSTAGI | KASHVI GUPTA | SHAGUN TOMAR

SETU AI | EXECUTIVE STRATEGY

Chennai first, Hyderabad next — a consent-first healthcare admin AI layer for a more inclusive and sustainable India.

SRI VENKATESWARA COLLEGE, DELHI

WHY SOUTH + HEALTHCARE?

5x
TN health cover ₹5L → ₹25L (Aug 2026)
Larger claims, more documentation and scrutiny.

~15 lakh
International medical tourists every year in Tamil Nadu
Every patient needs multilingual discharge, consent and insurance papers.

320 MW
Chennai data centre capacity (18% of India)
Sufficient for rented inference; no need to own a data centre.

DATA CENTRE CAPACITY CONCENTRATION (as of 2026)

Mumbai	Chennai	Rest of India
46%	18%	36%
846 MW	320 MW	~660 MW

MUMBAI WATER RISK (Jun 2026)

<7%

Reservoirs under 7%
10% cut, 20% for commercial.
100 MW DC ≈ 20 lakh L/day.

Our WUE ≤ 1.5 target is at risk.

MAHARASHTRA POWER STRAIN

27,230 MW
peak demand(Apr 2026, record)

AEML demand:
~50 → 638 MW by 2027.
Thermal-heavy mix vs our 70% clean target.

INSIGHT FUNNEL: WHY HEALTHCARE

All industries	Many AI opportunities, but need a trust layer (DPDP consent, audit).
Regulated AI buyers	Health data is sensitive under DPDP: consent is mandatory.
Healthcare	AI risk processes weak; South has a language gap (Tamil/Telugu).
Admin & claims	TN cover 5× → more claims, more denials.
Chennai hub	Concentrated buyers, reachable market, clear whitespace (no diagnosis).

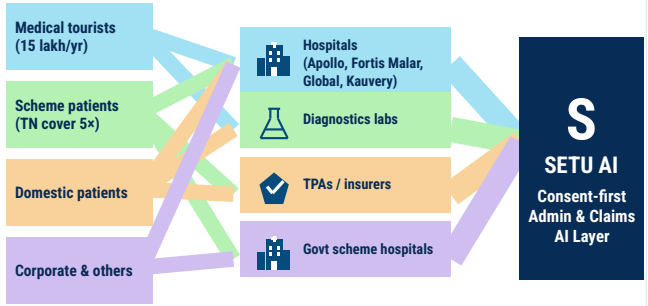
KEY FACTS THAT DRIVE THE CHOICE

Medical tourism scale
~15 lakh
International patients a year (TN)
So what? Every patient needs multilingual discharge, consent and insurance papers.

Insurance capital 5×
₹5L → ₹25L
CM scheme cover (Aug 2026)
So what? Larger claims mean more scrutiny and denials: direct Claims-Denial Copilot demand.

Concentrated buyers
Apollo, Fortis Malar, Global, Kauvery (in Chennai)
So what? Reachable buyers with open admin whitespace (5C Networks does imaging, not admin).

TARGET CUSTOMER FLOW (NEXT 24 MONTHS)



INITIAL CUSTOMER PRIORITY

1	2	3	4
Mid-size hospitals (200–600 beds)	Diagnostics labs	TPAs / insurers	Govt scheme hospitals
Claim denials + discharge summaries	Multilingual reports + consent	Claim review workload	5× cover, high volume (Year 2+)

Indicative sales cycle

Labs	Hospitals	TPAs	Corporate chains
3–5 months	4–6 months	6–9 months	9–12 months

SOUTH VS MUMBAI: KEY TRADE-OFFS

Mumbai wins on size, we win on forcing event, language gap and a better fit with our green targets.

	Chennai (South)	Mumbai (Maharashtra)
Data centre capacity	320 MW (18%)	846 MW (46%)
Water availability	Relatively stable	Reservoirs < 7% (Jun 2026)
Power mix	Better renewable mix	Peak 27,230 MW, thermal-heavy
Language need	Tamil / Telugu (gap)	Marathi / Hindi (less gap)
Policy trigger	TN cover 5× (Aug 2026)	More players, higher competition
Buyer concentration	Apollo, Fortis Malar, Global, Kauvery	Qure.ai, Wadhwani AI, Tata Memorial (imaging heavy)
Flood risk	Lower	Coastal DC parks, higher exposure
Fit with green targets	Easier (WUE ≤ 1.5)	At risk

WHAT SETU BUILDS (PHASED)

Chennai hub, Hyderabad spoke. Admin and claims first, no diagnosis (no clinical liability).

Phase 1 (0–6 months) Consent & Documentation
- Multilingual discharge summaries (Tamil/Telugu)
- Consent capture (DPDP)
- Insurance paperwork
- No diagnosis, no clinical liability.

Phase 2 (6–18 months) Claims-Denial Copilot
- Claim pre-check & coding
- Denial risk prediction
- TPA/hospital workflow integration
- Expand to more hospitals

Phase 3 (18–24 months) Scale & Expansion
- More states and languages
- Additional modules (e.g. patient support)
- Enter insurance and corporate chains
- Prepare for Mumbai entry (after 40% of pipeline comes from outside South)

DECISION: Chennai first → Hyderabad next → Mumbai only after 40% of pipeline comes from outside the South.

Build for scale, with trust, access and a lower footprint.

A Chennai-first healthcare GTM can convert claims friction into a scalable, partner-led business

SOUTH INDIA × HEALTHCARE | CUSTOMER ACQUISITION & 24-MONTH EXECUTION BLUEPRINT

SRI VENKATESWARA COLLEGE, DELHI

WHY SOUTH × HEALTHCARE?

SOUTH INDIA → HEALTHCARE ADMIN & CLAIMS → MEASURABLE BUYER URGENCY

₹5L ⟶ ₹25L
TN CMCHIS coverage increase announced August 2026

1h / 3h
IRDAI cashless pre-authorisation / final-authorisation decision timelines

~2,157 hospitals
Tamil Nadu CMCHIS empanelment (statewide, Jan 2025)

Concentrated healthcare demand plus time-sensitive claims workflows make Chennai a credible first sales hub.

Chennai first ⟶ Hyderabad expansion after product-market validation

WHO BUYS — AND HOW FAST?

1 PRIMARY BEACHHEAD
200–600-bed private multispecialty hospital
- Chennai / Tamil Nadu
- Buyer: Hospital CFO / COO
- Manual claims, slow approvals
- Setu: faster, cleaner claims with measurable ROI

Sales cycle 4–6 months | Pilot 8–12 weeks paid

2 ADJACENT EARLY ACCOUNT
Regional diagnostic laboratory network
- Chennai and other Tamil Nadu cities
- Buyer: COO / Operations Director
- High claim rejections, manual work
- Setu: automated claims processing and quicker reimbursement

Sales cycle 3–5 months | Pilot 8–12 weeks paid

3 STRATEGIC ENTERPRISE ACCOUNT
Health-insurance TPA / insurer claims-operations unit
- Hyderabad spoke after Chennai validation
- Buyer: Head of Claims / COO
- High document variability, manual QC
- Setu: scalable, compliant claims pre-processing

Sales cycle 6–9 months | Pilot 8–12 weeks paid

Customer journey (from first meeting to expansion)

Target account → Discover → Pilot → Validate ROI → Contract → Expand

₹50 CR | 24-MONTH EXECUTION BLUEPRINT

Workstream	H1: M1–6 Foundation	H2: M7–12 Paid pilots	H3: M13–18 Commercial expansion	H4: M19–24 Scale + optimisation	Total (₹ crore)
People & product	3.50	4.50	5.00	6.00	19.00
Compute & infrastructure	0.50	1.00	2.00	2.50	6.00
Sales & enterprise support	0.75	1.25	1.75	2.25	6.00
Marketing & demand creation	0.25	0.40	0.60	0.75	2.00
Data acquisition & localisation	1.25	1.25	1.00	0.50	4.00
Compliance & cybersecurity	1.25	0.80	0.55	0.40	3.00
Half-year spend (₹ crore)	**7.50**	**9.20**	**10.90**	**12.40**	**40.00**

₹10 CR RESERVE | UNDEPLOYED THROUGH BASE PLAN

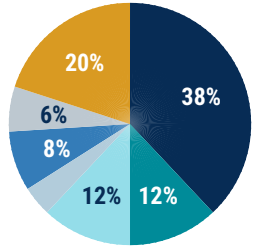
H1 (M1–6)
Product beta
2 hospital design partner pilots
HIS/EMR integration partners
Privacy architecture and MVP

H2 (M7–12)
Paid pilots
3–5 geographies targets
Initial productisation
First commercial annual hospital contract

H3 (M13–18)
Commercial expansion
5+ paying customers
Expand into Hyderabad after Chennai validation
Partner-led scale

H4 (M19–24)
Scale to profitability
10+ paying customers
State-wide TN ⟶ Hyderabad
Renewals, attach economics and brand and market awareness

₹50 CRORE BUDGET ALLOCATION



People & product delivery	₹19 cr	38%
Compute & infrastructure	₹6 cr	12%
Enterprise sales & integration support	₹6 cr	12%
Marketing & demand creation	₹2 cr	4%
Data acquisition & localisation	₹4 cr	8%
Legal, cybersecurity & compliance	₹3 cr	6%
Ring-fenced cash reserve	₹10 cr	20%
Total	**₹50 cr**	**100%**

HOW WE REACH BUYERS



DIRECT ENTERPRISE ABM
- Founder-led sales
- CFO/COO outreach
- Paid pilot ⟶ contract

HIS / EMR / LIS PARTNERS
- Partner integrations
- Joint GTM
- Embedded workflows

HEALTHCARE ECOSYSTEM REFERRALS
- Hospital networks
- Consultants & KOLs
- ROI workshops

EVIDENCE-LED INBOUND
- Case studies
- Customer references
- Targeted marketing

No big brand needed: earn trust through measurable paid pilots, then scale through hospital IT partnerships and verified customer referrals.

A focused product, defensible moats and green compute strategy can make Setu scalable

PRODUCT & MOAT | BUILD–BUY–BORROW | ENVIRONMENTAL STRATEGY

SRI VENKATESWARA COLLEGE, DELHI

WHAT SETU BUILDS — AND WHAT IT BUYS OR BORROWS

PRODUCT STACK: BUILD vs BUY/BORROW

SETU BUILDS | Core differentiated product layer

Workflow Copilot
Healthcare admin claims-readiness document intelligence

Consent & Audit Layer
Approvals traceability human review compliance logs

Hospital Ops Intelligence
Bed flow discharge coordination revenue-cycle signals

Integration Orchestration
HIS/EMR/LIS connectors workflow routing dashboards

SETU BUYS / BORROWS

Foundation models / APIs
Third-party or open-source models

Rented compute
Shared cloud IndiaAI-eligible GPU access no owned data centre

Infra tools
Vector DB monitoring security tooling analytics

Partner rails
HIS/EMR integrations implementation partners cloud vendors

ADVANTAGES & BUILD TIME

Workflow integration IP	Consent + audit architecture	Operational data loop
6–9 months	3–6 months	9–15 months

WEAK / COPYABLE WITHIN 12 MONTHS

- Generic multilingual copilot
- Standard dashboards
- Basic model-routing logic

Defensibility comes from workflow embedding, data discipline and operating fit — not from generic AI features.

OUR STRONGEST MOATS

PATIENT TWIN AI | CLINICAL INTELLIGENCE MOAT

Build horizon: 12–18 months

Historical cases → Treatment journeys → Similar-patient twin → Validated feedback → Stronger case-specific intelligence

- Accumulates clinical intelligence
- Improves with validated feedback
- Harder to replicate over time

POST-PAID, OUTCOME-LINKED AI | FINANCIAL MOAT

Build horizon: 2–4 months to launch; 6–9 months to mature

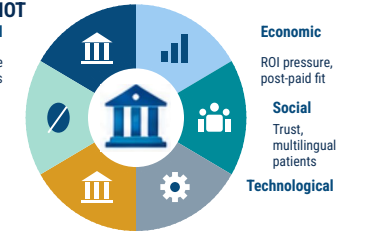
Low upfront risk → Pilot deployment → KPI measurement → Milestone billing → Aligned value + easier adoption

- Reduces adoption friction
- Aligns revenue with outcomes
- Creates stickier renewals

COPY RISK

- Commercial model is copyable
- Data-backed proof and trust compound more slowly

PESTLE SNAPSHOT



ENVIRONMENTAL STRATEGY — WHERE IT RUNS, WHAT IT IMPACTS

FINAL COMPUTE STRATEGY

Primary run environment
- Reliable cloud / rented GPU provider
- India-based or compliant region
- Healthcare data-residency aware
- No owned infrastructure

Workload design
- Small models for routine tasks
- Cache repeated queries
- Batch flexible jobs
- Larger models only when benefit is proven
- Carbon-aware scheduling for evaluations and batch processing

Why this setup
- Lower cost
- Lower energy intensity
- Operational flexibility
- Meets security and latency needs

ILLUSTRATIVE 24-MONTH IMPACTS

100,000 GPU-hours	70 MWh IT energy	98 MWh facility electricity	68.6 tCO_2e emissions	35,000 litres operational water
100,000 × 0.7 kW = 70 MWh (IT)	70 × 1.4 PUE = 98 MWh	98,000 kWh × 0.7 kg = 68.6 tCO_2e	70,000 kWh × 0.5 L = 35,000 L	

24-MONTH TARGETS

Energy/request	Water / 1,000 requests	Carbon / 1,000 requests
↓ 25%	↓ 15%	↓ 20%
vs baseline after 90-day measurement setup	vs baseline after 90-day measurement setup	vs baseline after 90-day measurement setup

COST vs SPEED vs ENVIRONMENT

Cheapest ≠ greenest	Fastest ≠ lowest-carbon	Largest model ≠ best-value

HOW WILL WE MANAGE?

Route → Batch → Cache → Measure → Report → Govern

KEY ISSUES TO MANAGE

- Cost volatility
- Water-data gaps
- Latency vs cleaner compute

APPENDIX A1 | DEFINING OUR NUMBERS

Calculation bridge, evidence status and management assumptions across slides 1–3

SRI VENKATESWARA COLLEGE, DELHI

[F] reported fact [D] derived arithmetic [A] team assumption [T] target | ₹ crore unless stated | Source IDs refer to Appendix A2

01 | CAPITAL ALLOCATION & PIE — SLIDE 2

Allocation [A]	₹ cr	Share [D]	Derivation
People / product	19.00	38%	19 ÷ 50
Rented compute	6.00	12%	6 ÷ 50
Sales / integration	6.00	12%	6 ÷ 50
Marketing	2.00	4%	2 ÷ 50
Data / localisation	4.00	8%	4 ÷ 50
Compliance / security	3.00	6%	3 ÷ 50
Cash reserve	10.00	20%	10 ÷ 50
TOTAL ENVELOPE	50.00	100%	40 spend + 10 reserve

Top-down allocations, not a headcount × salary cost build.
₹40 cr = 19 + 6 + 6 + 2 + 4 + 3; reserve is not expenditure.
No government subsidy or grant is assumed in the base plan.

02 | GANTT RECONCILIATION — SLIDE 2

Workstream	H1	H2	H3	H4	Total
People	3.50	4.50	5.00	6.00	19.00
Compute	0.50	1.00	2.00	2.50	6.00
Sales	0.75	1.25	1.75	2.25	6.00
Marketing	0.25	0.40	0.60	0.75	2.00
Data	1.25	1.25	1.00	0.50	4.00
Compliance	1.25	0.80	0.55	0.40	3.00
Spend [D]	7.50	9.20	10.90	12.40	40.00
Cash left [D]	42.50	33.30	22.40	10.00	–

H1–H4 = M1–6 / M7–12 / M13–18 / M19–24 [A].
Column totals = workstream sums; cash = 50 − cumulative spend.
No receipts assumed; ₹10 cr reserve intact at M24 [D].

03 | GTM, BUYERS & MILESTONES — SLIDES 1–2

Channel mix [A]	45 / 30 / 15 / 10% of qualified opportunities; ABM / IT partners / referrals / inbound. Per 100 = 45 / 30 / 15 / 10.
Basis / budget	Proposed mix, not measured conversion or spend share. ₹6 cr sales + ₹2 cr marketing = ₹8 cr enablement [D].
Buyer / sales [A]	200–600 beds; labs 3–5m, hospitals 4–6m, TPAs 6–9m, chains 9–12m; paid pilots 8–12 weeks.
H1–H2 goals [T]	H1: 2 hospital design partners + 1 HIS/EMR partner. H2: 3–5 geographies shown; stretch, subject to Chennai gate.
H3–H4 goals [T]	5+ / 10+ paying organisations, cumulative—not signed. Profitability is not derived: pricing, CAC and margins absent.
Expansion [T]	Hyderabad after validation; Mumbai only after >40% of pipeline is outside South. Count/value basis must be set.

04 | REGIONAL FACTS & SCOPE — SLIDES 1–2

Cover [F/D] · R1–2	₹25L ÷ ₹5L = 5× cover; +400% [D]. Announced Aug 2026; NOT evidence of 5× claims or denials.
TN scale [F/D] · R3	~15 lakh patients/year: minister-reported, not TAM. 942 government + 1,215 private = 2,157 hospitals (Jan 2025).
Capacity [F/D] · R4	846 + 320 + ~660 = ~1,826 MW reconstructed total. Shares ≈ 46.3 / 17.5 / 36.1% → 46 / 18 / 36%; rounded.
Water [F] · R5	6.93% reservoir stock (29 Jun 2026); 10% general / 20% industrial-commercial cuts are dated snapshots.
Power [F] · R6–7	27,230 MW = MSEDCL areas, not Maharashtra total; subsequently 28,303 MW. ~50 → 638 MW is a projection.
DC benchmark · R8	100-MW facility ≈ 2 million L/day = 20 lakh L/day; illustrative cooling benchmark, not Setu consumption.
Claims [F] · P1	1h pre-authorisation / 3h final authorisation apply to insurers; Setu supports hospital document readiness.

05 | ENVIRONMENTAL CALCULATION BRIDGE — SLIDE 3

Scenario inputs [A]	100,000 GPU-hours over 24m; 0.7 kW allocated IT power; PUE 1.4; grid 0.7 kg CO_2e/kWh; WUE 0.5 L/kWh IT.
IT energy [D]	100,000 h × 0.7 kW = 70,000 kWh = 70 MWh.
Facility energy [D]	70 MWh × 1.4 PUE = 98 MWh = 98,000 kWh.
Emissions [D]	98,000 kWh × 0.7 kg/kWh ÷ 1,000 = 68.6 tCO_2e.
Water [D]	70,000 kWh IT × 0.5 L/kWh = 35,000 L. Do not apply WUE to facility energy or apply PUE twice.
Cost illustration [A/D]	100,000 h × ₹65–92/h = ₹65–92 lakh; at hypothetical 40% subsidy: ×0.60 = ₹39–55.2 lakh. No award assumed.
Boundary / bridge	GPU scenario ≠ ₹6 cr total cloud envelope; APIs, CPU, storage/network not costed. Excludes embodied impacts. [M1–2]

06 | TARGETS, BUILD TIMES & CAVEATS — SLIDES 1–3

90-day baseline [T]	M24 intensity = baseline × (1 − reduction). Energy/request 0.75×; water/1,000 0.85×; carbon/1,000 0.80 ×.
Other targets [T]	WUE ≤1.5 L/kWh IT and ≥70% clean-energy share are provider-selection ambitions; scenario WUE is 0.5.
Supporting goals [T]	≥65% scheduled GPU utilisation where feasible; quarterly reporting for all metered workloads—not achievements.
Build horizons [A]	Workflow integration 6–9m; consent/audit 3–6m; operational data loop 9–15m; Patient Twin 12–18m.
Moat timing [A]	Post-paid model: 2–4m launch / 6–9m maturity. Generic copilots, dashboards, routing copyable within 12m.
Roadmap [A]	0–6 / 6–18 / 18–24m product phases; may overlap Gantt workstreams. Clinical twin requires separate validation.
Interpretation	Targets are not achieved savings; growth can raise total emissions. Post-paid terms do not themselves prove a moat.

APPENDIX A2 | REFERENCE REGISTER

Evidence, policy and competitor sources supporting the Setu AI case

SRI VENKATESWARA COLLEGE, DELHI

REFERENCE / WHAT IT SUPPORTS		
REGIONAL EVIDENCE & INFRASTRUCTURE		
R1 · CMCHIS ₹5L → ₹25L announcement, 19 Aug 2026	New Indian Express — announcement ↗	
R2 · H1 2026 capacity: Mumbai 846 MW / Chennai 320 MW; rounded shares	Savills — H1 2026 report ↗	Reported city-level figures ↗
R3 · Mumbai 6.93% reservoir stock and 10% / 20% water cuts; June 2026 snapshot	PTI / Rediff — dated water report ↗	
R4 · AEML ~50 → 638 MW by 2027: reported projection, not observed total demand	TechPolicy.Press — demand projection ↗	
R5 · 100-MW hyperscaler ≈ 2 million litres/day; cooling / location sensitivity	CEEW — data centres, power and water ↗	
POLICY, CLAIMS, PRIVACY & COMPUTE		
P1 · IRDAI cashless timelines: 1h pre-authorisation / 3h final authorisation	PIB — official claims-timeline summary ↗	ABDM components* ↗
P2 · Tamil Nadu Safe & Ethical AI Policy 2020: responsible deployment framework	Tamil Nadu government — policy PDF ↗	
P3 · DPDP Rules / commencement: consult phased dates, not blanket immediate duties	MeitY — rules and enforcement timeline ↗	PIB notification ↗
P4 · SPDI Rules 2011: medical records, security and processing safeguards	WIPO Lex — rules text ↗	
Additional official compute resource	IndiaAI — official compute portal ↗	
COMPETITOR & ADJACENT-ECOSYSTEM LANDSCAPE		
C1 · IHX: direct overlap in claims infrastructure, document and discharge workflows	IHX — payer solutions ↗	IHX — provider solutions ↗
C2 · Yellow.ai: healthcare chatbots, appointment and support workflows	Yellow.ai — healthcare ↗	
C3 · Wadhwani AI: social-impact and public-health AI; not solely an imaging competitor	Wadhwani AI — official platform ↗	
C4 · Tata Memorial Centre: hospital / research ecosystem, not a like-for-like software vendor	Tata Memorial Centre — official site ↗	
ENVIRONMENTAL METHODS & BOUNDARIES		
M1 · Energy efficiency, carbon-aware workloads and software carbon-intensity methodology	Green Software — principles ↗	SCI specification ↗
M2 · AI lifecycle, water and resource impacts; broader than operational emissions alone	UNEP — lifecycle issue note ↗	UNEP — sustainable data centres ↗

*ABDM components blocked automated retrieval (403); retained as an official navigation link. A1 assumptions and calculations are team-authored, not validated by these websites.